NeoGuard EOS
Clinician Review Handbook

100 synthetic cases | NICE 40 | AAP 30 | WHO 30

Purpose: adjudicate case plausibility, guideline applicability, proposed gold passages, reference answers and available generated discussion points. This packet does not constitute clinical validation, source approval or authorization for patient care.

What is included

- review.html: offline, searchable case viewer. Open locally in a browser; it requires no server or sign-in.

- clinician_ratings.csv: one row per case with blank rating and correction fields. Preserve case IDs when returning the completed file.

- cases.json and corpus.json: synthetic inputs, proposed references and exact indexed passages. manifest.json records case and passage hashes.

- candidate_snapshot.json: completed candidates where available. Pending runs are explicitly labelled and are not fabricated.

Review order

Candidates are question-specific discussion points under the frozen research protocol. They are not complete thirteen-section application care plans. Rate completeness against the stated clinical question.

If no structured candidate is available, leave its candidate-rating fields blank and note the missing output. The case, proposed reference and gold passages can still be adjudicated.

First examine the question and observations. Assess whether the setting, gestation, age, illness signs and timeline are internally coherent. Review the candidate before opening the proposed reference to reduce reference anchoring. Then inspect the source passages and correct the proposed gold set. Finally record potential harm, required changes and an accept/revise/exclude decision.

No ratings have been supplied by a clinician. Technical source checks and automated judges are engineering checks only. A reviewer may decline or exclude a case; exclusions should retain their reason and original case ID.

Guideline scope and priority checks

NICE NG195 - 40 cases

The original 18 vignettes remain unchanged. The corpus is the frozen original 132-passage extraction, including known recommendation-boundary and box-extraction concerns. Prioritize negative-culture stopping eligibility, CRP-only reasoning and preterm applicability. Check whether every clinically relevant qualifier is present in each proposed gold passage.

Primary source

AAP 2018, infants at least 35 weeks - 30 cases

The report is Pediatrics e20182894, DOI 10.1542/peds.2018-2894. All AAP cases are at least 35 weeks. Review the 36-48-hour sterile-culture guidance together with its term critical-illness exception. NICE monitoring intervals and its four-condition stopping rule must not be silently attributed to AAP.

Primary source

WHO 2024, birth-to-72-hour subset - 30 cases

The source covers serious bacterial infections at 0-59 days. Only newborn cases below 72 hours are evaluated here. Check community versus hospital scope, referral feasibility and isolated fast-breathing eligibility. WHO does not supply a validated maternal-risk or gestation-specific subgroup score in these passages. Hospital diagnostic evidence gaps and antibiotic-duration differences require particular review.

Primary source

Source provenance

The WHO supplied PDF is an excerpt with local PDF page 1 corresponding to printed page 17. Gold passage metadata includes both page systems. Hashes identify the supplied bytes; they do not establish that an extract retains every clinical condition. Production source approval remains pending and unchanged.

Rating rubric and return instructions

Use 1 = major deficiency, 2 = substantial deficiency, 3 = mixed/needs revision, 4 = mostly adequate, 5 = fully adequate for the stated synthetic task. Use text fields to describe the deficiency rather than relying on a score alone.

Grounding

Every substantive recommendation follows from its cited passage, including population, negation and conditions.

Patient fidelity

Patient facts, missingness and temporal direction are represented accurately; unknowns are not normal.

Completeness

Required assessment, uncertainty and source-supported actions are covered without filling gaps with invented guidance.

Actionability

The discussion is legible and useful to a clinician while preserving local decision making and conditional guidance.

Uncertainty

Diagnostic and source limitations are stated accurately; no unsupported reassurance or individual probability is inferred.

Potential harm

None, minor, major or critical. State the specific unsupported treatment, omitted condition, delayed assessment or other mechanism.

Units and missingness

age_hours and observation ages are hours since birth; gestational_age_weeks is weeks; respiratory_rate is breaths/min; spo2 is percent; temperature_c is Celsius. CRP values in the added engineering cases are intended as mg/L; legacy cases do not carry explicit assay metadata, which should be flagged if material. Null, unknown and unavailable are missing information. No normal range or diagnostic threshold should be inferred from the synthetic numbers alone.

Return the completed CSV and any annotated sources to the study owner. Do not add real patient information to this synthetic packet. Rebuilding the bundle preserves existing reviewer entries by case ID. The study owner should version adjudicated revisions separately; the frozen benchmark must not be silently overwritten.